

SINGAPORE GENERAL HOSPITAL

DISCHARGE SUMMARY

PATIENT PARTICULARS:

Name: Tan Wei Ming
NRIC/FIN: S8765432A
Date of Birth: 1975-08-15
Age: 49 years old
Gender: Male
Contact No: 9123 4567
Address: Blk 123, Bedok North Ave 1, #04-567, Singapore 460123

ADMISSION DETAILS:

Admission Date: 2025-04-10, 14:30
Discharge Date: 2025-04-14, 11:00
Ward: Ward 56A
Attending Physician: Dr. Chong Wei Liang (MCR-10234A)

DIAGNOSES:

Primary: Pneumonia, unspecified organism (ICD-10: J18.9)

PROCEDURES/INTERVENTIONS:

1. Subsequent hospital inpatient visits (CPT 99232)
2. Chest X-ray, 2 views (CPT 71046)

CHIEF COMPLAINT:

Fever and cough for 3 days.

HISTORY OF PRESENT ILLNESS:

Mr. Tan Wei Ming, a 49-year-old gentleman, presented to the Emergency Department (ED) on 2025-04-10 with a 3-day history of fever, productive cough, and generalized body aches. He reported intermittent fevers up to 39.0°C, associated with chills and rigors. His cough was initially dry, but became productive with yellowish sputum on the day of admission. He denied any shortness of breath, chest pain, hemoptysis, or recent travel history. He had self-medicated with over-the-counter paracetamol with temporary relief. No known sick contacts. Appetite was poor, but oral intake was maintained.

PAST MEDICAL HISTORY:

- Hypertension, controlled on Amlodipine 5mg OD.
- Hyperlipidemia, controlled on Simvastatin 20mg ON.
- No known drug allergies (NKDA).

SOCIAL HISTORY:

- Non-smoker, occasional social alcohol use.
- Lives with wife and two children in a HDB flat.
- Works as an administrative executive.

PHYSICAL EXAMINATION (On Admission, 2025-04-10):

General: Conscious, alert, comfortable, no acute distress.
Vitals: T 38.5°C, HR 98 bpm, BP 130/80 mmHg, RR 20 breaths/min, SpO2 96% on room air.
Respiratory: Mild tachypnoea. Chest expansion symmetrical. Coarse crepitations noted over the right lower lung field. No wheezes or rhonchi.

Cardiovascular: Dual heart sounds, no murmurs. Capillary refill <2 seconds.
Abdomen: Soft, non-tender, no organomegaly. Bowel sounds present.
Extremities: No clubbing, cyanosis, or oedema.

INVESTIGATIONS (On Admission):

Blood Tests:

- Full Blood Count: WBC $14.5 \times 10^9/L$ (Neutrophils 85%), Hb 14.2 g/dL, Platelets $280 \times 10^9/L$.
- C-Reactive Protein (CRP): 150 mg/L (elevated).
- Renal Panel: Urea 5.2 mmol/L, Creatinine 88 $\mu\text{mol/L}$, eGFR >90 mL/min.
- Liver Function Test: Within normal limits.
- Procalcitonin: 0.8 ng/mL.
- Blood Culture: Pending (subsequently reported as no growth).

Imaging:

- Chest X-ray (2025-04-10): Right lower lobe consolidation, consistent with pneumonia. No pleural effusion or cardiomegaly.

HOSPITAL COURSE:

Mr. Tan was admitted to Ward 56A under the care of Dr. Chong Wei Liang for community-acquired pneumonia.

Upon admission, he was initiated on intravenous (IV) Amoxicillin/Clavulanate 1.2g TDS and supportive care, including IV fluids, antipyretics (Paracetamol), and nebulized salbutamol as needed for cough relief.

Over the next 48 hours, Mr. Tan showed good clinical improvement. His fever subsided on 2025-04-11 and remained afebrile. His cough improved significantly, becoming less frequent and less productive. Oxygen saturation remained stable at 97-98% on room air.

Serial vital signs were stable, and physical examination on 2025-04-12 showed reduced crepitations over the right lower lung field.

Blood tests on 2025-04-13 showed a downtrend in inflammatory markers (WBC $9.8 \times 10^9/L$, CRP 45 mg/L).

Given his sustained clinical improvement, stable vital signs, and normalizing inflammatory markers, Mr. Tan was deemed suitable for discharge on oral antibiotics to complete his course. He was counselled on his condition and discharge medications.

DISCHARGE MEDICATIONS:

1. Amoxicillin/Clavulanate (Augmentin) 625mg: 1 tablet twice daily for 7 days. (To complete 10-day course including IV).
2. Paracetamol 500mg: 1-2 tablets every 6 hours as needed for fever or pain.
3. Amlodipine 5mg: 1 tablet once daily (continue usual medication).
4. Simvastatin 20mg: 1 tablet once nightly (continue usual medication).

DISCHARGE INSTRUCTIONS AND ADVICE:

- Continue all prescribed medications as instructed. It is crucial to complete the full course of antibiotics even if feeling better.
- Maintain good hydration and nutrition.
- Avoid strenuous activities for the next 1-2 weeks. Gradual return to normal activities.
- Practice good hand hygiene and cough etiquette.
- Monitor for any worsening of symptoms such as increasing fever, shortness of breath, chest pain, or persistent cough.
- Return to the Emergency Department immediately if any of these warning signs occur.

FOLLOW-UP:

- Follow-up at Bedok Polyclinic on 2025-04-21, 09:00 AM for review of pneumonia and chronic conditions.
- Please bring this discharge summary and all current medications to your follow-up appointment.

Dr. Chong Wei Liang
Attending Physician

MCR-10234A

Singapore General Hospital
Date: 2025-04-14